**جوستاف روسي انترناشيونال - مصر (EPE) *** **طلب علاج أول مرة / متردد**

**إسم المريض رباعياً :** احمد معتوق على عماره
**المحافظة :** القاهره **الحي / المدينة :** المطريه **العنوان :** ----
**رقم البطاقة :** 25601250101137 **رقم التليفون :** 01275929771 **تاريخ تسجيل الإستمارة :** 23-10-2026
**العمر(تاريخ الميلاد) :** 25-01-1956 **المهنة :** ربه منزل

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان القنوات المراريه
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان القنوات المراريه
**خطة العلاج :** أورام كيماوي علاج تدعيمي وتلطيفي [6396] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

**Name:** احمد معتوق على عماره
**National ID:** 25601250101137

## تقرير طبي

**السيد**

**تحية طيبة وبعد,,**

| | |
|---|---|
| **إيماء للطلب المقدم من** | **بشأن تقرير طبي للمريض** |
| **بقسم:** | **رقم طبي:** |

**نتشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | |
|---|---|
| **تاريخ وساعة الدخول :** -- --:-- yyyy-mm-dd | **تاريخ وساعة الخروج :** PM 01:17 05-07-2026 |

**وبعد الكشف والفحوصات والمتابعة وجد انة**

**A patient of bile duct cancer for Supportive and palliative care with follow-up**

| | |
|---|---|
| **التاريخ :** **05-07-2026** | **الوقت :** **-- --:--** |
| | **الطبيب المعالج :** د/شيماء لاشين |

196.219.52.214/checkin/Reports.aspx

الهيئة العامة للتامين الصحي
الادارة المركزية لنظم المعلومات و التحول الرقمي

HEALTH INSURANCE ORGANIZATION
CUNTER DEPARTMENT OF
INFORMATION SYSTEM AND DIGITAL
TRANSFORMATION

رسوم الاستفادة 20 جنيه

تاريخ الاصدار :10-8-2026

جهه الاصدار :فرع القليوبية

الى من يهمه الامر .....

الهيئه العامة للتامين الصحي تعدل من خلال قوانين منظمة للانتفاع الخاضعين لقانوني التامين الاجتماعي رقم 79 لسنه 1975 المعدل بالقانون 25 لسنه 1977 "القوى العامله والمعاشات والارامل " و كذلك قانون 99 لسنه 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنه 2012 " الاطفال دون السن المدرسي" وقانون رقم 23 لسنه 2012 الخاص بالمرأة المعيله .

وطبقا لقاعدة البيانات المرسلة من التامينات الاجتماعيه

وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي

و البحث تبين أن

السيد (ة) احمد معروف علي بطاقة رقم 25601230101137

تليفون رقم: 01010349399

وطبقا الى خطاب من التامينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الغاء غير المنتفعه بالتامين الصحي صدر هذا الخطاب دون ادنى مسئولية على الهيئة العامة للتامين الصحي حال خضوع المواطن لقانون التامين الصحي الشامل
وشكرا

تاريخ الطباعه :10-8-2026

ادارة مركز نظم المعلومات
بخدمتك...

مسلسل 2046824

الخطاب صالح لمدة ثلاثة اشهر من تاريخ الاصدار ولا يعتد به في حالة تعديل من قبل الهيئة العامة للتامينات الاجتماعية او وزارة التضامن الاجتماعي او وزارة التربية و التعليم

الادارة المركزية لنظم المعلومات و التحول الرقمي

Non-HIO Letter
[[DOCTYPE:NON_HIO_LETTER]]




-١٤٧٨٨٨٢٤٩٥




National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

| | |
|---|---|
| **Name:** احمد معتوق على عماره | |
| **MR. :** 13663 | |

Detailed Medical Report

**السيد**

**تحية طيبة وبعد,,**

| | | | |
|---|---|---|---|
| **إيماء للطلب المقدم من** | | **بشأن تقرير طبي للمريض** | |
| **بقسم:** | | **رقم طبي:** | |

**ننشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | | | |
|---|---|---|---|
| **تاريخ وساعة الدخول :** | | **تاريخ وساعة الخروج :** | |

**وبعد الكشف والفحوصات والمتابعة وجد انه**

**The patient is a 70-year-old individual with no significant medical or family history. The patient presented with right hypochondrial pain. A CT scan demonstrated gallbladder mural thickening and a lesion in the left hepatic lobe. A biopsy of the left hepatic lobe lesion revealed a moderately differentiated adenocarcinoma. Tumor markers were obtained, although specific values are not provided in the note.**

**تحريرا في ,**

| | | | |
|---|---|---|---|
| **التاريخ :** | **19-09-2026** | **الوقت :** | |

**الطبيب المعالج :** نرمين مصطفي عبد القادر بهي الدين

**يعتمد ,**

الهيئة العامة
لمستشفيات والمعاهد التعليمية
مستشفى المطرية التعليمي
قسم الاشعة

Name Ahmed Maatouq Ali
Date Wednesday, August 19, 2026

**a pelvi-abdominal U/S reveals**

- **Liver:** is average in size with homogenous parenchymal echopattern. No intra-hepatic biliary radicle dilatation. **Left lobe shows well defined iso to hypoechoic focal lesion measuring about 2.8 x 2.1 cm, taking no internal vascularity on applying color Doppler.**
- **PV & CBD (both are patent and not dilated).**
- **Gall bladder:** is average in size **shows biliary sludge with increased wall thickness (6mm). no stones**
- **Both kidneys:** are within average size and normal site and shape. Normal parenchymal thickness with preserved cortico-medullary differentiation, **yet shows increased parenchymal echogenicity bilaterally.** No stones or back pressure changes on either side. **Right kidney shows simple cortical cyst measuring about12.6 x 8 cm.**
- **Rt kidney: 8.7 x 4.6 x 1.9 cm**
- **Lt kidney: 10.9 x 5 x 1.3 cm**
- **Spleen:** is average in size with homogenous texture and no focal lesions. Splenic vein is not dilated.
- **Pancreas and para-aortic area (visualized part):** are masked by gases.
- **Urinary bladder:** is suboptimally distended with urine hindering proper pelvic examination.
- No fluid collection.

**CONCLUSION:**

- **Left hepatic focal lesion for Triphasic Ct abdomen and pelvis.**
- **GB med with thickened wall.**
- **Bilateral grade I nephropathy.**
- **Right renal simple cyst.**

*Much obliged,*

*Dr, Mayada Adel*

تم المسح الضوئي بـ CamScanner
تم المسح الضوئي بـ CamScanner
Ultrasound
[[DOCTYPE:ULTRASOUND]]

مركز النور المحمدى للأشعة
Alnour Almohammady Radiological Center
رنين مغناطيسى مفتوح - أشعة مقطعية - موجات صوتية - دوبلر ملون
22502699 - 22517341 - 26509762 : ت
مسجد النور المحمدى - ميدان المسلة
Alnour Almohammady Masqid - Masalla Square
Tel.: 26509762 - 22517341 - 22502699

**PATIENT'S NAME:** *Ahmad Maatouq*
**DATE:** *15/7/2026*
**CODE:**2

## C.T REPORT

## MULTISLICE C.T SCAN OF WAS THROUGH THE ABDOMEN AND PELVIC WITH ORAL AND I.V. CONTRAST INJECTION

- ***Gall bladder sizable irregular dense stone is noted measuring about 2 cm with relative distended gall bladder and irregular wall thickening more evident along its fundus with marked stranded peri cholecystic fat planes extending to para colic fat along right colonic flexure that also seen diffuse thickened with stranded nodular reaction of surrounding fat planes***
- *The hepatic surfaces are rather lobular with slightly heterogeneous parenchyma mainly at the gall bladder bed.*
- *The spleen is of average size with grossly intact parenchyma.*
- *Right renal sizable cortical cyst is noted measuring about 12x10 cm otherwise, both kidneys are of average size with normal parenchymal thickness and no obstructive uropathy could be detected.*
- *Good contrast concentration and excretion of both kidneys.*
- *Normal appearance of the pancreas, aorta, IVC and both adrenal glands.*
- *Normal CT appearance of the urinary bladder with no stones, masses or diverticular changes*
- *No ascites or lymphadenopathy could be detected.*

## IMPRESSION:

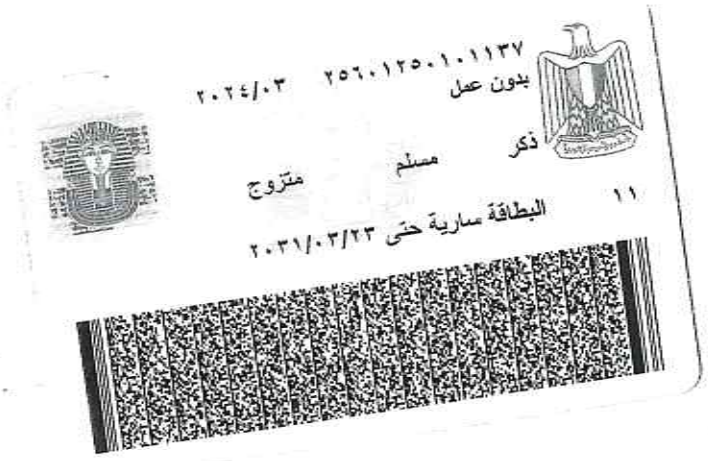
- *Gall bladder stones are noted with marked irregular mural soft tissue thickening extending to para colic aspect as well as right colonic flexure mural thickening as described above suggestive of ?? inflammatory reaction for clinical correlation and follow up.*
- *Right renal sizable simple benign looking cortical cyst.*

*YOURS SINCERELY*
***DR. MOHAMED FOUAD***
*PROF. DR OF RADIODIAGNOSIS*
***DR/ MAHMOD EL AWADY, MSC***
*AL-AZHAR UNIVERSITY*

CT Scan
[[DOCTYPE:CT_SCAN]]

[[DOCTYPE:ID_FRONT_BACK]]
National ID – Front & Back



[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report



GUSTAVE ROUSSY

| | | | |
|---|---|---|---|
| **Patient Name** | **احمد معتوق على** | **Case No.** | **384721** |
| **Sex/Age** | **Female 70 Y** | **Patient No.** | **27751** |
| **Registered Date** | **15/9/2026 09:35:05 AM** | **Referred By Dr.** | **Out Patient** |
| **Validation Date** | **19/9/2026 11:36:02 AM** | **Branch** | **Gustave Roussy** |
| **Reporting Date** | **24/9/2026 10:16:00 AM** | **Pathology No.** | **SR-1033/26** |

# Pathology Report

**Clinical Data:** Hepatic focal lesion in a patient with gall bladder irregular mural thickening.

**Nature of specimen:** Hepatic focal lesion, Referred block & slide (Tru-cut biopsy).

**Gross Examination:**
One referred block & one referred (H&E) stained slide labelled by (26H044) and patient Arabic name.

**Technique:**
- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for; CK7 clone (SP52) & CK20 (SP33).

**Microscopic Examination**
Histopathological examination reveals cores of tumor formed of malignant epithelial cells displaying eosinophilic cytoplasm & enlarged moderately pleomorphic nuclei, arranged in trabeculae & fused glands with single disperesed cells amid desmoplastic stroma. There is focal necrosis.

** **IPT (Control show appropriate reactivity):**
- CK7: Positive in the malignant cells.
- CK20: Negative.

**Diagnosis**
**Hepatic focal lesion, Referred block & slide (Tru-cut biopsy):**
- Moderately differentiated adenocarcinoma (Positive CK7).
- Differential diagnosis includes; Upper gastro-intestinal tract (GIT) versus biliary tract origin, as well as other (CK7 Positive) tumors.
- Case for clinical & radiological data correlation.

**Signature**
Prof. Dr. Maha Guimei, M.D, PhD.

الهيئة العامة
لمستشفيات والمعاهد التعليمية
مستشفى المطرية التعليمي
قسم الاشعة

Name : Ahmed Maatouq Ali
Date : Wednesday, August 19, 2026

**¤ pelvi-abdominal U/S reveals**

- **Liver:** is average in size with homogenous parenchymal echopattern. No intra-hepatic biliary radicle dilatation. **Left lobe shows well defined iso to hypoechoic focal lesion measuring about 2.8 x 2.1 cm, taking no internal vascularity on applying color Doppler.**
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- **Urinary bladder:** is suboptimally distended with urine hindering proper pelvic examination.
- No fluid collection.

**CONCLUSION:**

- **Left hepatic focal lesion for Triphasic Ct abdomen and pelvis.**
- **GB med with thickened wall.**
- **Bilateral grade I nephropathy.**
- **Right renal simple cyst.**

*Much obliged,*

*Dr, Mayada Adel*

تم المسح الضوئي بـ CamScanner

[[DOCTYPE:OTHER_COLONOSCOPY]]
colonoscopy

The General Organization For teaching Hospital
And inistitutes
Ahmed Maher Teaching Hospital
Advanced Gastrointestinal Endoscopy Unit

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
وحده مناظير الجهاز الهضمى المتقدمه

# Colonoscopy Report

**Name:** احمد معتوق علي
**ID:** 22412
**Date:** Sunday 30, August 2026
**Age:** 70

**Indications**
Screening for malignancy.

**Medications**
Midazolam and Nalbuphine

**Instrument**
Olympus GIF 160

**Procedure Technique**
After informed consent was obtained and a physical exam was performed, the patient was connected to the monitoring device and put in the left lateral position. Colonoscope introduced in to the rectum and advanced under direct visualization. The scope was subsequently removed slowly while examining the color, texture, anatomy, and integrity of the mucosa on the way out. The patient was subsequently transferred to the recovery area in satisfactory condition. The following findings were noted:

**Findings**

Boston preparation(7/9)
Colonscopy up to the terminal ileum were done and shows no masses or polyps nor ulcers apart from three internal piles grade II-III have been seen on retroflexion.

**Conclusion**
Internal piles.

**Recommendation**
Upper endoscopy is recommended.

**Endoscopist :** Dr Omar Azzam MD

٣٤١ ش بورسعيد باب الخلق القاهره الدور التالت فاكس ٢٣٩١٤٤٥٥ & ت ٠١٢٠٢١٢٤٦٧٨

مركز النور المحمدى للأشعة
Alnour Almohammady Radiological Center
رنين مغناطيسى مفتوح - أشعة مقطعية - موجات صوتية - دوبلر ملون
ت : 22502599 - 22517341 - 26509782
مسجد النور المحمدى - ميدان المسلة
Alnour Almohammady Mosque - Mesalla Square
Tel.: 26509782 - 22517341 - 22502599

***PATIENT'S NAME: Ahmad Maatouq***
***DATE: 15/7/2026***
***CODE:2***

## C.T REPORT

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## IMPRESSION:

- *Gall bladder stones are noted with marked irregular mural soft tissue thickening extending to para colic aspect as well as right colonic flexure mural thickening as described above suggestive of ?? inflammatory reaction for clinical correlation and follow up.*
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*YOURS SINCERELY*
***DR. MOHAMED FOUAD***
*PROF. DR OF RADIODIAGNOSIS*
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22502599 - 22517341 - 26509782 :
Alnour Almohammady Mosque - Mesalla Square
Tel.: 26509782 - 22517341 - 22502599

Dear collage:

Re: MR: Ahmed Maatouq

Thank you for reference.

The above mentioned patient has significantly distended gall bladder with mural thickening and surrounding soft tissue thickening of the fat planes including the paracolic fat planes along the right colic flexure.

Hypodense hepatic areas of low density is also noted at the gall bladder bed with small focal hepatic lesions.

The central intrahepatic biliary radicles are also slightly dilated.

The right hypochondrium region has been also encroached upon by large simple renal cyst (about 12x10cm).

The appearances might be attributed to (? Chronic inflammatory reaction) (? Infiltrative lesion) for clinical correlation and follow up triphasic study guided by the ultrasound guided biopsy pathological result.

YOURS SINCERELY
**DR. MOHAMED FOUAD**
PROF. DR OF RADIODIAGNOSIS
AL-AZHAR UNIVERSITY



جمهورية مصر العربية
بطاقة تحقيق الشخصية

احمد
معتوق على عماره
١٣ ش مسجدالرحمن
الخصوص - القليوبية

٢٥٦٠١٢٥٠١٠١١٣٧

١٩٥٦/٠

K00069371



٢٥٦٠١٢٥٠١٠١١٣٧ ٢٠٢٤/٠٣
بدون عمل

ذكر مسلم متزوج

١١ البطاقة سارية حتى ٢٠٣١/٠٣/٢٣





GUSTAVE ROUSSY

| | | | |
|---|---|---|---|
| **Patient Name** | احمد معتوق على | **Case No.** | **384721** |
| **Sex/Age** | **Female 70 Y** | **Patient No.** | **27751** |
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**Signature**
Prof. Dr. Maha Guimei, M.D, PhD.

الهيئة العامة
لمستشفيات والمعاهد التعليمية
مستشفى المطرية التعليمي
قسم الاشعة

Name : Ahmed Maatouq Ali
Date Wednesday, August 19, 2026

**¤ pelvi-abdominal U/S reveals**

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**CONCLUSION:**

- **Left hepatic focal lesion for Triphasic Ct abdomen and pelvis.**
- **GB med with thickened wall.**
- **Bilateral grade I nephropathy.**
- **Right renal simple cyst.**

*Much obliged,*

*Dr, Mayada Adel*

تم المسح الضوئي بـ CamScanner

The General Organization For teaching Hospital
And inistitutes
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Advanced Gastrointestinal Endoscopy Unit

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
وحده مناظير الجهاز الهضمى المتقدمه

# Colonoscopy Report

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**Findings**

Boston preparation(7/9)
Colonscopy up to the terminal ileum were done and shows no masses or polyps nor ulcers apart from three internal piles grade II-III have been seen on retroflexion.

**Conclusion**
Internal piles.

**Recommendation**
Upper endoscopy is recommended.

**Endoscopist :** Dr Omar Azzam MD

٣٤١ ش بورسعيد باب الخلق القاهره الدور التالت فاكس ٢٣٩١٤٤٥٥ & ت ٠١٢٠٢١٢٤٦٧٨

مركز النور المحمدى للأشعة
Alnour Almohammady Radiological Center
رنين مغناطيسى مفتوح - أشعة مقطعية - موجات صوتية - دوبلر ملون
مسجد النور المحمدى - ميدان المسلة ت : 26509782 - 22517341 - 22502599
Alnour Almohammady Mosque - Mesalla Square Tel.: 26509782 - 22517341 - 22502599

***PATIENT'S NAME: Ahmad Maatouq***
***DATE: 15/7/2026***
***CODE:2***

## C.T REPORT

## MULTISLICE C.T SCAN OF WAS THROUGH THE ABDOMEN AND PELVIC WITH ORAL AND I.V. CONTRAST INJECTION

- ***Gall bladder sizable irregular dense stone is noted measuring about 2 cm with relative distended gall bladder and irregular wall thickening more evident along its fundus with marked stranded peri cholecystic fat planes extending to para colic fat along right colonic flexure that also seen diffuse thickened with stranded nodular reaction of surrounding fat planes***
- *The hepatic surfaces are rather lobular with slightly heterogeneous parenchyma mainly at the gall bladder bed.*
- *The spleen is of average size with grossly intact parenchyma.*
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- *Good contrast concentration and excretion of both kidneys.*
- *Normal appearance of the pancreas, aorta, IVC and both adrenal glands.*
- *Normal CT appearance of the urinary bladder with no stones, masses or diverticular changes*
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## IMPRESSION:

- *Gall bladder stones are noted with marked irregular mural soft tissue thickening extending to para colic aspect as well as right colonic flexure mural thickening as described above suggestive of ?? inflammatory reaction for clinical correlation and follow up.*
- *Right renal sizable simple benign looking cortical cyst.*

*YOURS SINCERELY*
***DR. MOHAMED FOUAD***
*PROF. DR OF RADIODIAGNOSIS*
***DR/ MAHMOD EL AWADY, MSC***
*AL-AZHAR UNIVERSITY*

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Dear collage:

Re: MR: Ahmed Maatouq

Thank you for reference.

The above mentioned patient has significantly distended gall bladder with mural thickening and surrounding soft tissue thickening of the fat planes including the paracolic fat planes along the right colic flexure.

Hypodense hepatic areas of low density is also noted at the gall bladder bed with small focal hepatic lesions.

The central intrahepatic biliary radicles are also slightly dilated.

The right hypochondrium region has been also encroached upon by large simple renal cyst (about 12x10cm).

The appearances might be attributed to (? Chronic inflammatory reaction) (? Infiltrative lesion) for clinical correlation and follow up triphasic study guided by the ultrasound guided biopsy pathological result.

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جمهورية مصر العربية
بطاقة تحقيق الشخصية

احمد
معتوق على عماره
١٣ ش مسجدالرحمن
الخصوص - القليوبية

٢٥٦٠١٢٥٠١٠١١٣٧

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بدون عمل

ذكر مسلم متزوج

١١ البطاقة سارية حتى ٢٠٣١/٠٣/٢٣





GUSTAVE ROUSSY

| | | | |
|---|---|---|---|
| **Patient Name** | احمد معتوق على | **Case No.** | **384721** |
| **Sex/Age** | **Female 70 Y** | **Patient No.** | **27751** |
| **Registered Date** | **15/9/2026 09:35:05 AM** | **Referred By Dr.** | **Out Patient** |
| **Validation Date** | **19/9/2026 11:36:02 AM** | **Branch** | **Gustave Roussy** |
| **Reporting Date** | **24/9/2026 10:16:00 AM** | **Pathology No.** | **SR-1033/26** |

# Pathology Report

**Clinical Data:** Hepatic focal lesion in a patient with gall bladder irregular mural thickening.

**Nature of specimen:** Hepatic focal lesion, Referred block & slide (Tru-cut biopsy).

**Gross Examination:**
One referred block & one referred (H&E) stained slide labelled by (26H044) and patient Arabic name.

**Technique:**
- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for; CK7 clone (SP52) & CK20 (SP33).

**Microscopic Examination**
Histopathological examination reveals cores of tumor formed of malignant epithelial cells displaying eosinophilic cytoplasm & enlarged moderately pleomorphic nuclei, arranged in trabeculae & fused glands with single disperesed cells amid desmoplastic stroma. There is focal necrosis.

** **IPT (Control show appropriate reactivity):**
- CK7: Positive in the malignant cells.
- CK20: Negative.

**Diagnosis**
**Hepatic focal lesion, Referred block & slide (Tru-cut biopsy):**
- Moderately differentiated adenocarcinoma (Positive CK7).
- Differential diagnosis includes; Upper gastro-intestinal tract (GIT) versus biliary tract origin, as well as other (CK7 Positive) tumors.
- Case for clinical & radiological data correlation.

**Signature**
Prof. Dr. Maha Guimei, M.D, PhD.

الهيئة العامة
لمستشفيات والمعاهد التعليمية
مستشفى المطرية التعليمي
قسم الاشعة

Name : Ahmed Maatouq Ali
Date Wednesday, August 19, 2026

**¤ pelvi-abdominal U/S reveals**

- **Liver:** is average in size with homogenous parenchymal echopattern. No intra-hepatic biliary radicle dilatation. **Left lobe shows well defined iso to hypoechoic focal lesion measuring about 2.8 x 2.1 cm, taking no internal vascularity on applying color Doppler.**
- **PV & CBD (both are patent and not dilated).**
- **Gall bladder:** is average in size shows **biliary sludge with increased wall thickness (6mm). no stones**
- **Both kidneys:** are within average size and normal site and shape. Normal parenchymal thickness with preserved cortico-medullary differentiation, **yet shows increased parenchymal echogenicity bilaterally.** No stones or back pressure changes on either side. **Right kidney shows simple cortical cyst measuring about12.6 x 8 cm.**
- **Rt kidney: 8.7 x 4.6 x 1.9 cm**
- **Lt kidney: 10.9 x 5 x 1.3 cm**
- **Spleen:** is average in size with homogenous texture and no focal lesions. Splenic vein is not dilated.
- **Pancreas and para-aortic area (visualized part):** are masked by gases.
- **Urinary bladder:** is suboptimally distended with urine hindering proper pelvic examination.
- No fluid collection.

**CONCLUSION:**

- **Left hepatic focal lesion for Triphasic Ct abdomen and pelvis.**
- **GB med with thickened wall.**
- **Bilateral grade I nephropathy.**
- **Right renal simple cyst.**

*Much obliged,*

*Dr, Mayada Adel*

تم المسح الضوئي بـ CamScanner

The General Organization For teaching Hospital
And inistitutes
Ahmed Maher Teaching Hospital
Advanced Gastrointestinal Endoscopy Unit

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
وحده مناظير الجهاز الهضمى المتقدمه

# Colonoscopy Report

**Name:** احمد محتوق علي
**ID:** 22412
**Date:** Sunday 30, August 2026
**Age:** 70

**Indications**
Screening for malignancy.

**Medications**
Midazolam and Nalbuphine

**Instrument**
Olympus GIF 160

**Procedure Technique**
After informed consent was obtained and a physical exam was performed, the patient was connected to the monitoring device and put in the left lateral position. Colonoscope introduced in to the rectum and advanced under direct visualization. The scope was subsequently removed slowly while examining the color, texture, anatomy, and integrity of the mucosa on the way out. The patient was subsequently transferred to the recovery area in satisfactory condition. The following findings were noted:

**Findings**

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Colonscopy up to the terminal ileum were done and shows no masses or polyps nor ulcers apart from three internal piles grade II-III have been seen on retroflexion.

**Conclusion**
Internal piles.

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**Endoscopist :** Dr Omar Azzam MD

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[[DOCTYPE:CT_SCAN]]
CT Scan

**Medical No. :** 10795
**Patient's Name:** Ahmed Matouq Ali Imarah
**Date:** 2026-10-11
**Referred by:** Prof.Dr. no doctor
**Hospital:** Goustave Roussey International Egypt

***MULTI-SLICE NON-CONTRAST CT SCAN OF THE CHEST REVEALED:***

***A Known case of infiltrative GB mass with hepatic metastatic deposits:***

- ***Mild right side pleural effusion with underling basal subsegmental consolidation collapse.***
- ***Bilateral multiple well defined calcified pulmonary nodules more evident at the left side …. Likely old granuloma.***
- ***Other subpleural and fissural based minute nodules …. Likely insignificant. for follow up.***
- ***Right apical fibrotic bands.***
- ***Bilateral basal atelectatic bands.***
- ***Bilateral few areas of mosaic lung attenuation …likely breath related.***
- *Patent **trachea**, main and segmental bronchi.*
- *No left pleural effusion.*
- *Normal **cardiac size**, no pericardial effusion.*
- *No detected significant enlarged hilar or mediastinal lymph nodes.*
- ***Degenerative changes of the scanned bones**, No aggressive bony lesions.*

- ***The scanned upper abdomen shows**: Infiltrative soft tissue lesion centered on the GB fossa with infiltration of the related segment IV of the liver.*
- ***Multiple suspicious looking bilobar hepatic focal lesions.***
- ***Diffuse thickening of the scanned peritoneal reflection with haziness and stranding of mesenteric fat planes associated with few peritoneal deposits….Suggestive of Peritoneal disease.***

***Ibrahim***

[[PATHOLOGY_REPORT]]
Pathology Report

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